

LAKESIDE IMAGING ASSOCIATES
Ordering Provider Note + Imaging Order Justification

Patient: Ramirez, Carlos
DOB: 1968-09-21 MRN: LIA-0457829
Date of Service: 11/15/2023
Ordering Provider: Sandra Goldfarb, MD (Neurology, NPI 1769988775)
Performing Facility: Lakeside Imaging Associates
Examination Ordered: MRI brain with and without IV contrast (CPT 70553)
ICD-10 Indications: G43.709 (Chronic migraine, unspecified), R51.9 (Headache),
H53.13 (Sudden visual loss, bilateral)

CLINICAL HISTORY / INDICATION FOR IMAGING

55-year-old male referred from neurology with progressive change in headache character over 8 weeks. Reports new-onset position-dependent headaches waking him at 4-5 AM, worse with Valsalva, partial relief upright. Two episodes of transient binocular visual blurring lasting 3-5 minutes accompanying severe headaches in the last 3 weeks. No photophobia, phonophobia, or aura that match his historical migraine pattern.

PRIOR WORKUP

- 10/02/2023: Comprehensive metabolic panel, CBC, ESR, CRP – all unremarkable. CRP 1.2 mg/L.
- 10/11/2023: Office neuro exam (Goldfarb) – papilledema noted bilaterally on funduscopic exam (Frisén grade 2). Documented in note.
- 10/18/2023: Outpatient CT head without contrast – no acute hemorrhage or large mass effect, but small right frontal hypodensity, "cannot exclude underlying lesion; recommend MRI with contrast."
- 10/30/2023: Trial of sumatriptan/topiramate ineffective for current symptom pattern.

WHY MRI WITH AND WITHOUT CONTRAST IS MEDICALLY NECESSARY

Per the ACR Appropriateness Criteria (Headache, Variant 6 – chronic headache with new features/papilledema), MRI brain with and without contrast is the most appropriate initial advanced imaging when CT findings are equivocal and papilledema is present. The combination of papilledema, position-dependent morning headaches, and indeterminate CT focus on the prior study raises clinical concern for an intracranial mass lesion or idiopathic intracranial hypertension; contrast is required to characterize enhancement patterns and exclude small neoplasm or meningeal disease. Imaging is being performed prior to escalating empiric pharmacologic management.

ATTESTATION

The ordering provider has reviewed and signed the order. Imaging is clinically indicated and not for screening purposes.

Electronically signed: Goldfarb, Sandra MD on 11/14/2023 17:08